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Outlines.

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❖ Definition

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Outlines.

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- ◆ Types of triage
- ◆ Triage Systems:

***Functions of triage system

*** Types of triage system:

- ◆ Triage Staffing

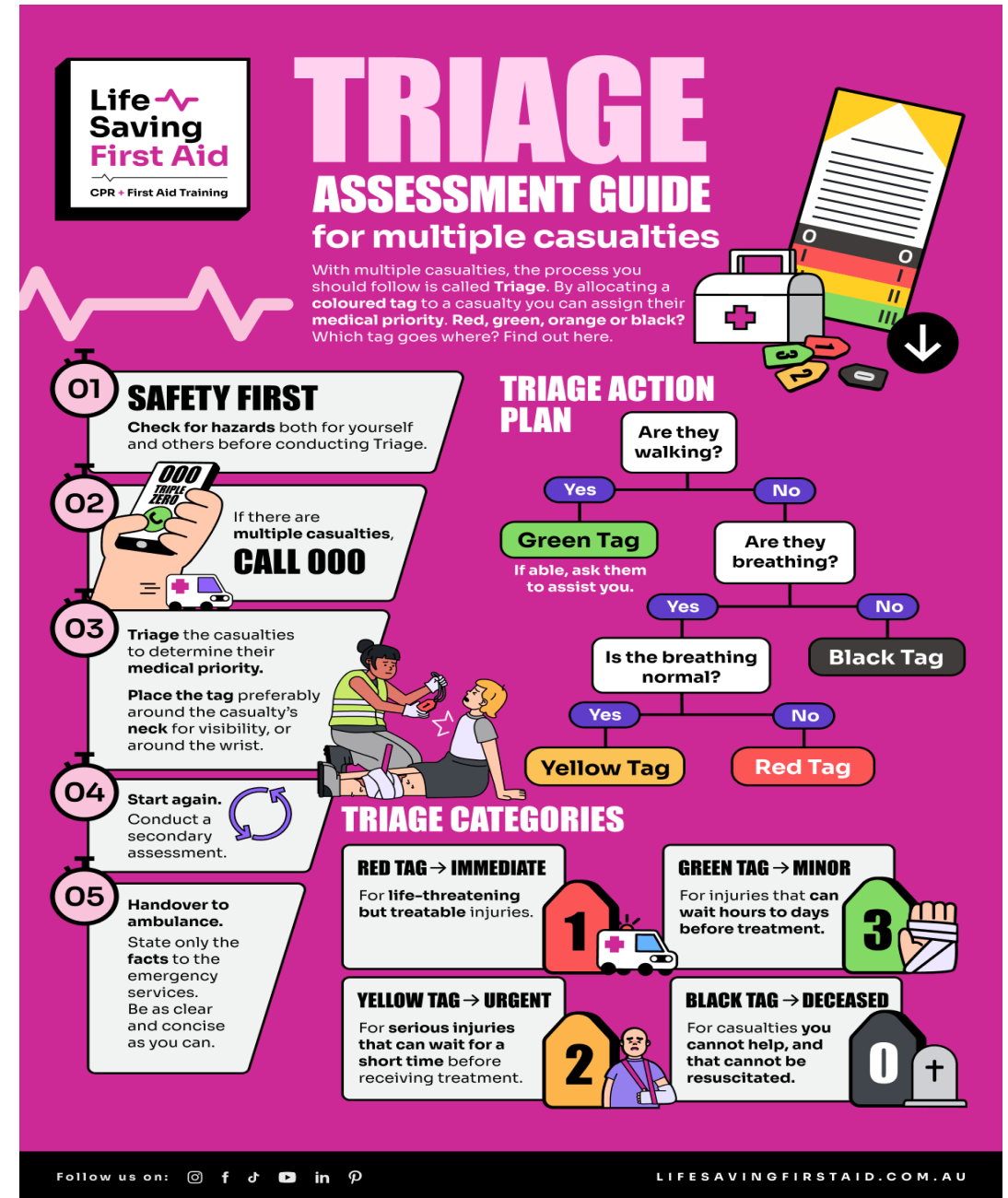
Introduction:

Triage is typically the first step in the evaluation of a patient presenting to an emergency department (ED). This process involves **a brief assessment** that focuses on the **patient's clinical needs** and **priority for care**. The triage nurse then assigns the patient a place in queue and to an appropriate treatment area of the ED based on that assessment.



Definition:

Triage is a process of sorting or classifying patients based on severity of illness or injury for each patient who enters the emergency department (ED).



Goals of Triage:

The primary goals of **an effective triage** system are

- Rapid identification of patients with urgent, life-threatening conditions.
- Prioritizing care needs for all patients
- Ensure that treatment is appropriately and timely
- Regulating patient flow through the ED.
- Avoid unnecessary congestion in emergency treatment areas.



Advantages of triage:

Benefits of triage include:

For the patients:

- ❑ Decrease patient's waiting time.
- ❑ The positive feelings by patients that they have received attention shortly after arrival.
- ❑ Decrease patient's treatment time.



Advantages of triage:

Benefits of triage include:

For the health care team:

- Ensure quality of health care
- Assure safety, security and appropriate condition for health care providers



Location and structure of triage area:

The triage area should be located by **the front door**, the triage area should be **large enough** to allow privacy for the interview without interfering with the triage nurse's ability to see patients .

- **Telephone** is needed for consultation, and referrals. Privacy should be maintained.
- **A stretcher and wheel chair** should be available in or adjacent to the triage area.
- **Equipment** for immediate emergency treatment i.e. airway, bag, mask, etc should be available.
- **A sink** for hand washing should be available.



Location and structure of triage area



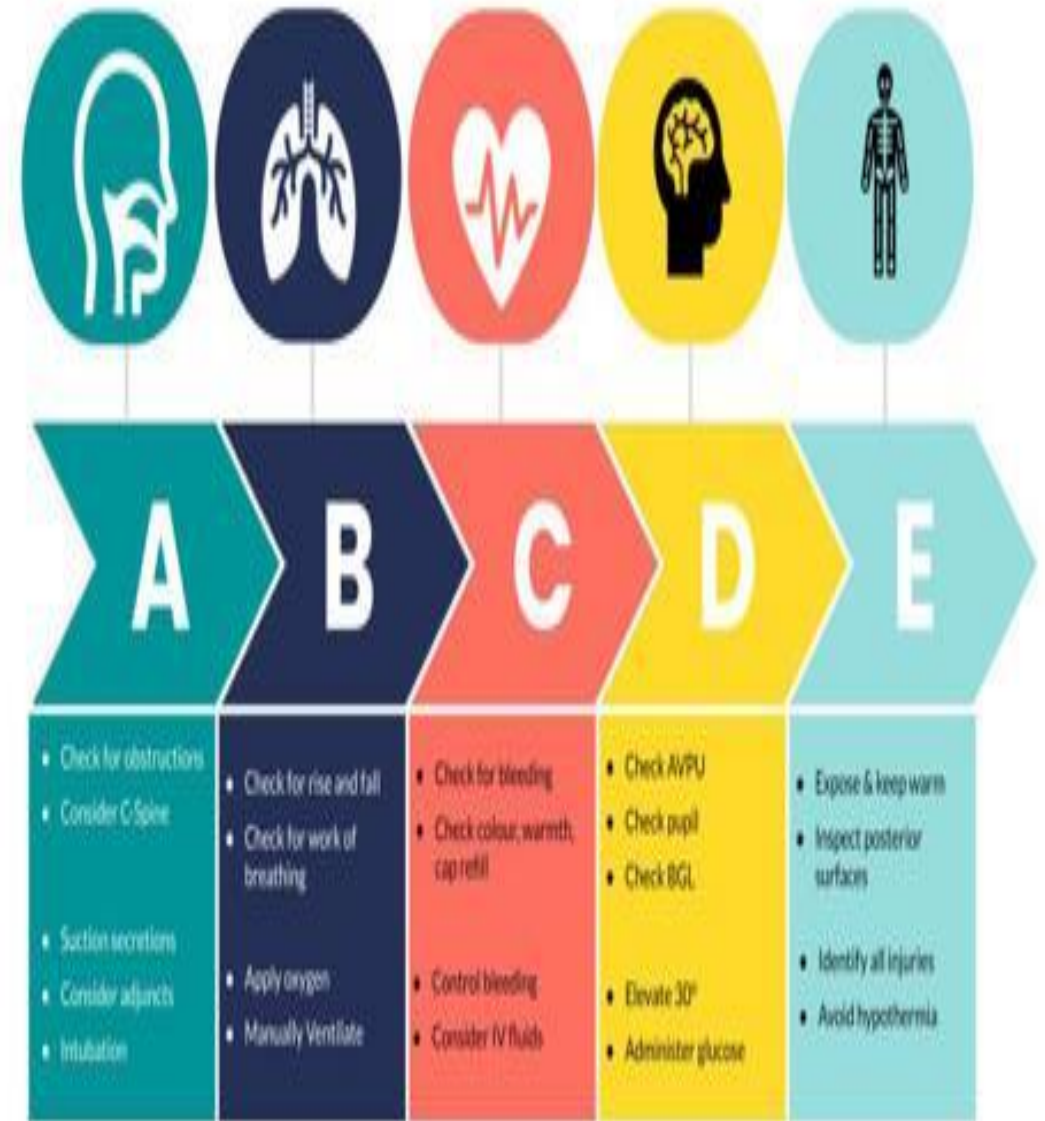
General Considerations of Triage:

- 1- Greet patients, identify our self and explain what we are doing.
- 2- Maintain patient confidentiality.
- 3- Try to visualize all incoming patients even while interviewing others.
- 4- Maintain the flow of information between triage personnel and patients in the waiting area.
- 5- Excellent communication is essential.
- 6 - Know the institution's system and its own limitations.
- 7- Keep in mind the primary objectives of the triage role and use resources to maintain the appropriate standard of care.



General Considerations of Triage:

- 8- Formal process of immediate assessment of patients presenting to ED, in addition to, ongoing process of continuous assessment and reassessment to promote patient safety via accurate assessment, prioritizing and provision of intervention.
- 9- Finally, classifies patients into groups according to severity of illness or injury.



Factors affecting on triage:

1. Number of ED visits per day.
2. Times of peak activity.
3. Number of qualified triage staff and availability of consultation staff.
4. Presence of a walk in clinics or other treatment areas outside the ED.
5. Logistic and administrative concerns.

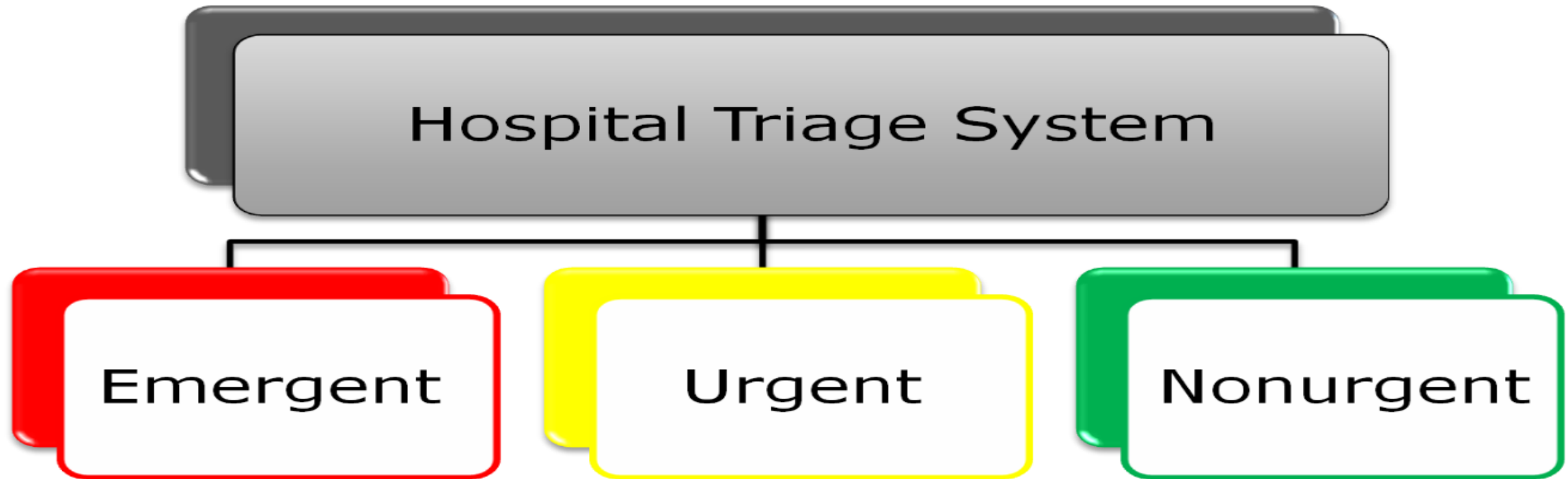
Triage priority Levels (methods):

A. Triage consist from Three level

B. Triage consist from Four level:

C. Triage consists from Five level

A.Triage consist from Three level



I) Emergent Categories

Definition	Time needed	Example
Those that are life threatening , but likely to be amenable to rapid intervention that does not require an inordinate amount of resources .	requires treatment immediately or within 15 to 30 minutes .	• Cardiac arrest . • Airway obstructions . • Hemorrhage with shock . • Complicated delivery. • Seizure • Asthma • A cute bleeding or acute pain . • Depressed level of consciousness.

II) Urgent Categories

Definition	Time needed	Example
Those conditions that are serious and if not treated in timely manner , are likely to deteriorate to become critical .	serious illness or injury that must be attended to, but wait of up to 2 hours	• Complex long bone fracture . • Bleeding controlled with a pressure dressing. • A cute psychiatric problem • High fever with other vital signs stable

III) Nonurgent Categories

Definition	Time needed	Example
conditions that can wait without likelihood of deterioration .	conditions that can wait more than 2 hours.	- Minor laceration which require sutures. - Minor joint trauma which requires an X- ray for diagnosis . - Request for medication refills. - Chronic conditions that are stable Such as preexisting skin rash

Triage consist from Four level:

The triage system is mainly used by health professionals in Disasters

- 1. Category I (emergent) (red):** Used for viable victims with potentially life threatening conditions. compromised airway, shock hemorrhage
- 2. Category II urgent (yellow):** Used for victims with non-life threatening injuries, but who urgently require treatment. open fracture, chest wound
- 3. Category III nonurgent (green):** Used for victims with minor injuries that do not require ambulance transport. minor injuries; walking wounded—closed fracture, sprain, strain
- 4. Category 0 expectant (black):** Used for victims who are dead, or whose injuries make survival unlikely.

C. Triage consists from Five level:

Most common triage systems consist of five levels of acuity as in

All patients presenting to emergency departments should be assessed in order to determine whether their condition requires medical intervention to avoid unnecessarily adverse outcomes within:

Levels

Urgency

1. Resuscitation

Immediately

2. Emergency

Minutes

3. Urgent

An hour

4. Acute

Hours

5. Non-urgent

Days

1) Triage Level I "*Resuscitation*"

Conditions requiring immediate nursing and physician assessment and any delay in treatment is potentially life- or limb-threatening.

Conditions includes such as

- Airway compromise
- Severe shock
- Multisystem trauma
- Eclampsia

Cardiac arrest

Cervical spine injury

- unconscious

**Foreign
Body Airway
Obstruction**

Jeff Kenneally



3) Triage Level III *"Urgent"*

Conditions that are required nursing and physician assessment within 30 minutes from arrival.

Conditions include

- Head injury with vomiting
- Moderate asthma
- Moderate trauma
- Abuse or neglect

4) Triage Level IV *"Less Urgent"*:

Conditions requiring nursing and physician assessment within one hour from arrival.

Conditions include

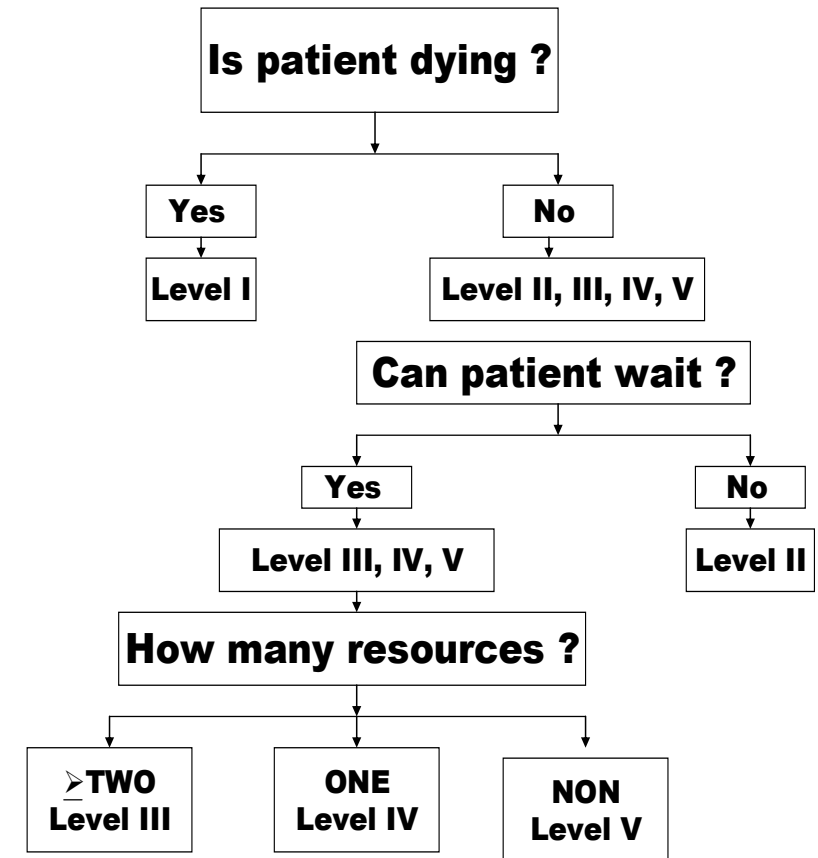
- head injury without vomiting
- Minor trauma
- Vomiting and diarrhea in patient older than age 2 without dehydration
- Earache

Global overview about patient condition by using The Emergency Severity

Index (ESI) (hospital triage):

The Emergency Severity Index (ESI) is an initial measure of patient assessment in the emergency department (ED). It rates patients based on acuity and predicted resource intensity from Level 1 (most ill) to Level 5 (least resource intensive). Already implemented in several US hospitals

ESI
M E N
E V D
R E E
G R X
E I
N T
C Y
Y



Types of triage:

1- Simple triage

2- Advanced triage

3- Reverse triage

4- Telephone Triage

Types of triage:

1- Simple triage:-

is usually used in a **scene of a "mass-casualty incident"** (is any incident in which emergency medical services personnel and equipment at the scene are overwhelmed by the number and severity of casualties as in such as earthquakes, hurricanes and accidents.)

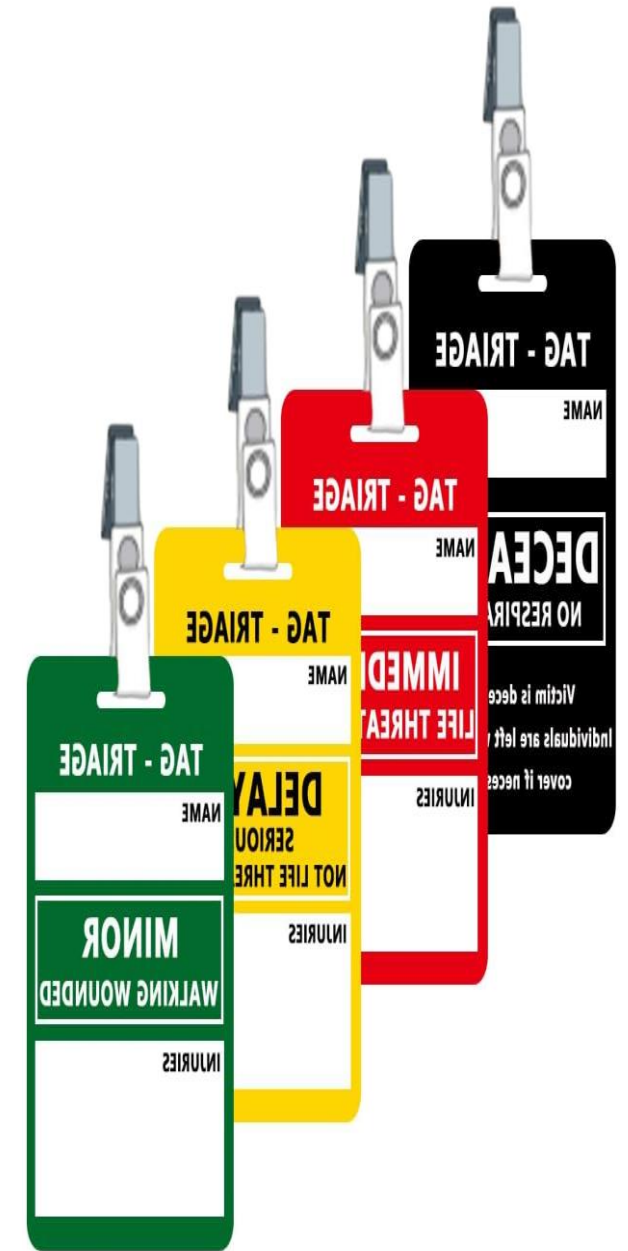


1- Simple triage:-

Use of printed [triage tags](#) (Triage Tags is a tool first responders and medical personnel use during a mass casualty incident).

Aim of the simple triage:-

- ❑ The first-arriving personnel are able to effectively and efficiently distribute the limited resources
- ❑ Provide the necessary immediate care for the victims until more help arrives or colored [flagging](#)



Colored flagging (Colored non-adhesive tape used in marking objects)

A color-coding scheme using red, yellow, green, white, and black tags:

- ◆ **Red tags** - (immediate) are used to label those who cannot survive without immediate treatment but who have a chance of survival.
- **Yellow tags** - (observation) for those who require observation (and possible later re-triage). Their condition is stable for the moment and, they are not in immediate danger of death. These victims will still need hospital care and would be treated immediately under normal circumstances.



Colored flagging **Colored non-adhesive tape used in marking objects**

- ◆ **Green tags** - (wait) are reserved for the "walking wounded" who will need medical care at some point, after more critical injuries have been treated.
- ◆ **White tags** - (dismiss) are given to those with minor injuries for whom a doctor's care is not required.
- ◆ **Black tags** - (expectant) are used for the deceased and for those whose injuries are so extensive that they will not be able to survive given the care that is available

2- Advanced triage:-

- ❖ In advanced triage, doctors may decide that some seriously injured people should not receive advanced care because they are unlikely to survive.
- ❖ Advanced care will be used on patients with less severe injuries.

2- Advanced triage:-

- ❖ Advanced triage has ethical implications. It is used to divert scarce resources away from patients with little chance of survival in order to increase the chances of survival of others who are more likely to survive.
- ❖ This has happened in disasters such as volcanic eruptions, and rail accidents. In these cases some percentage of patients will die regardless of medical care because of the severity of their injuries. Others would live if given immediate medical care, but would die without it.



3- Reverse triage

- ✓ This is a system in which the less wounded are given preferential treatment over other more serious patients.
- ✓ This type of system is common on the battlefields where the slightly wounded soldiers are quickly treated and sent back into the fray or in situations where physicians or other essential caregivers are among the wounded in which case, they are quickly treated so that they can recover quickly and assist the other caregivers.
- ✓ Wounded physicians receive priority treatment in reverse triage systems.



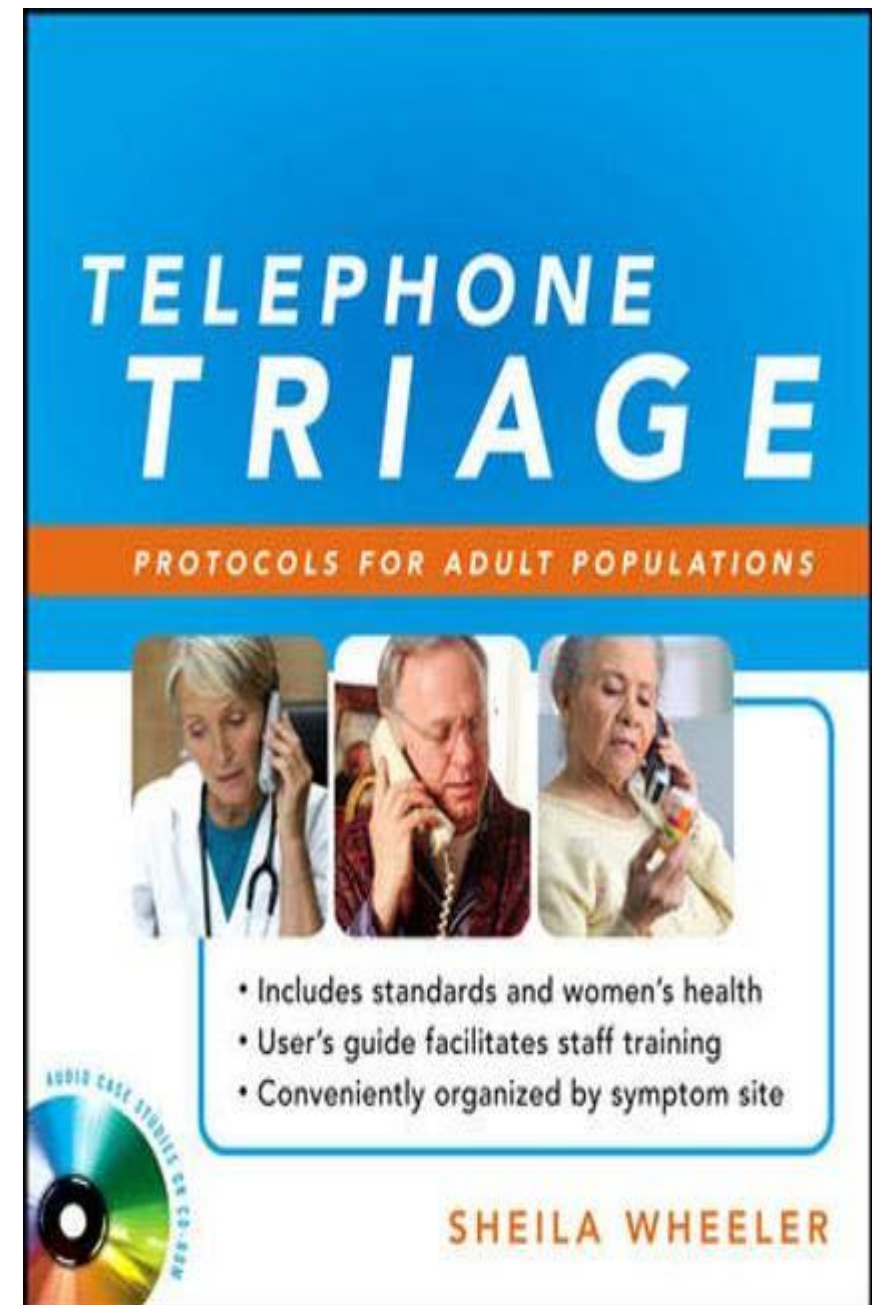
4- Telephone Triage:

Telephone calls eliciting medical information and advice are problematic for EDs since evaluation of patients by phone is difficult if not impossible. Most legal experts do not recommend giving advice and suggest the caller be directed to the ED.



Telephone Triage:

- ❖ In situations such as cardiopulmonary resuscitation or childbirth, giving appropriate instructions until emergency medical personnel arrive may positively affect the outcome.
- ❖ Hospitals providing telephone information need standardized protocols and a mechanism for documentation so that every call can be logged, tape-recorded, or both.
- ❖ Some EDs are providing special services to meet this consumer need for information.
- ❖ Trained nurses dedicated to telephone consultation using personal computers, specially designed software, telephone recording systems, and facsimile machines create an opportunity for EDs to strengthen their medical advice programs for the community.



Triage system

A triage system is a method for assessing patients to prioritize medical care based on the severity of their condition, rather than their arrival time.

Functions of triage system:

Any triage system has, primary and secondary functions.

1) Primary functions include assessment and reassessment of the patient's chief complaint and related symptoms, taking a brief history, physical assessment, and measuring vital signs.



***Functions of triage system:

1)Secondary functions include clerical tasks, directions, telephone advice, ambulance patient evaluation, ambulance dispatch, stocking of supplies, cleaning, equipment maintenance, crowd control, security, and information.

A triage system encompasses all these functions depends on the daily census, available staff, presence of walk-in clinics or same-day clinics, type and availability of health care providers, availability of specialty treatment areas, and environmental, legal, and administrative constraints.



Types of triage system:

1- Traffic director (Type I) triage system: in type I system nonprofessionals (i.e. receptionists, clerks or allied health personnel) are the first persons to interact with the patient. Their assessment is cursory. They determine the chief complaint and classify the patient as **"sick" or "not sick"** on the basis of appearance. Patients are the directed to the waiting area. Documentation is sparse or absent, and no diagnostic procedures are initiated.



Types of triage system:

2- Spot checker (Type II) triage system: in this system is done by a registered nurse or a physician. Both subjective and objective data related to the chief complaint are evaluated and documented, and an **urgency rating is assigned**. Initiation of diagnostic procedures in this system may be inconsistent and varies with the institution.

1) Patients may be directed to a specific care area or they may even be treated and released by triage physician.

Types of triage system:

1) Comprehensive (Type III) triage system: this triage is the most sophisticated of triage. This system includes the major components of the type II system i.e. assessment is performed by a professional, acuity ratings based on care assigned , and patients are directed to a specific care area or waiting room.

1) The comprehensive system distinguish from other systems by:

- The data base is expanded to include not only subjective and objective information about the chief complaint but also health care and learning needs of the patient.
- Documentation is consistent and ongoing
- Ordering of diagnostic procedures is consistent and based on protocols
- A schedule for periodic reassessment of patients waiting for treatment is provided as the following:

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Reassessment in triage

- Level 1 =Continuous
- Level 2 = every 15 min
- Level 3 = every 60 min
- Level 4 = every 60 to 90 min
- Level 5 = every 2 hours

Triage Staffing:

❑Historically

Receptionists

❑Ward clerks

Nurse aides

❑ Emergency medical technicians

❑Licensed practical nurses,

❑Registered nurses,

Physicians

❑Others performed triage are considered triage staff.

Registered nurse

A registered nurse to provide medical screening and prioritize patient care is widely recommended.

◆ Personal abilities that are be an effective triage nurse:

- Clinically experience.
- Good judgment and leadership.
- Calm and cool under stress.
- Decisive.
- Knowledgeable of available resource.
- Sense of humor.
- Creative problem solver.
- Availability.
- Experienced and knowledgeable regarding anticipated casualties



Thank
You

